

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 36

Age: 49 years old | Sex: Female

MRN: 000-TEST-36

Admission Date: [DATE] | Discharge Date: [DATE]

Length of Stay: 3 days

****PRIMARY DIAGNOSIS****

Osteoarthritis, right hip — s/p total hip arthroplasty (THA) with cemented acetabular component and cementless femoral stem

****SECONDARY DIAGNOSES****

- Hypothyroidism on levothyroxine replacement
- Generalized anxiety disorder, stable on current regimen
- Mild postoperative anemia (Hgb 9.2 g/dL on POD#2)

****HOSPITAL COURSE****

Patient 36 is a 49-year-old female admitted electively for right THA secondary to end-stage degenerative joint disease. Preoperative labs and imaging unremarkable. Orthopedic surgery performed standard right THA on POD#0 using cemented acetabular component (Zimmer-Biomet, 54 mm) and cementless femoral stem (press-fit design). Operative time 68

minutes; EBL ~350 mL. Intraoperative findings: significant cartilage loss, osteophyte formation, no intraarticular loose bodies.

Postoperative course uncomplicated. Patient extubated in OR, transferred to ICU for brief monitoring (4 hours), then to orthopedic floor. Pain controlled with epidural analgesia POD#0–#1, then transitioned to oral acetaminophen + oxycodone. Vital signs stable; SpO2 consistently >95% on room air. Sequential compression devices applied for DVT prophylaxis; enoxaparin 40 mg SC q.d. initiated POD#1 (planned 14-day course). Foley catheter removed POD#1 without difficulty. Wound inspection POD#2 showed clean, dry incision with intact staples; no erythema or drainage. PT evaluated POD#1; patient ambulated 35 feet with walker, TTWB right lower extremity per surgeon protocol. OT consulted POD#2 for ADL evaluation and home safety assessment.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Acetaminophen	650 mg	Q6H PRN pain/fever	4 weeks
Oxycodone	5–10 mg	Q4–6H PRN moderate–severe pain	2 weeks, then reassess
Enoxaparin (Lovenox)	40 mg	SC q.d.	14 days post-discharge
Levothyroxine	75 mcg	q.d. (take on empty stomach)	Ongoing
Sertraline	50 mg	q.d. at bedtime	Ongoing
Docusate (Colace)	100 mg	BID	4 weeks (stool softener with opioid use)

****ACTIVITY & WEIGHT-BEARING STATUS****

- ****Weight-Bearing:**** TTWB (Toe Touch Weight Bearing) right hip for 4 weeks post-discharge, then reassess with surgeon; advance to PWB (Partial Weight Bearing) if

tolerating well and pain controlled

- ****Hip Precautions — STRICT adherence required:****

- NO hip flexion >90 degrees (avoid low chairs, bending at waist)
- NO internal rotation of right hip (keep knee/toes pointing outward or neutral)
- NO crossing legs or bringing knee toward chest
- Sleep with pillow between legs × 4